

Orthopedic Driven Imaging

AP/PA Annotation Definitions

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Company: Orthopedic Driven Imaging, LLC

Contents

1	Preamble	2
2	Medical Approval	4
3	Image Annotations	5
3.1	Ilium Contour	5
3.2	Pelvic Outlet Contour	6
3.3	Pelvic Teardrops	7
3.4	Ischial Tuberosities	8
3.5	Pubic Symphysis Contour	9
3.6	Distal Sacroiliac Joint	10
3.7	Acetabular Contour	11
3.8	Femoral Head Contour	12
3.9	Greater Trochanter Contour	13
3.10	Proximal Medial Femur Contour	14
3.11	Lesser Trochanter Contour	15
3.12	Distal Medial Femur Contour	16
3.13	Obturator Foramen Contour	18
4	Annotation Order	19
5	General Guidance for Contour Annotation	22
5.1	What Is a Contour?	22
5.2	How to Annotate a Contour	22
5.3	Partly Obscured or Off-Screen Contours	22
6	Derived Features of Interest	24
7	Categorical Annotation	34
A	Annotation ID Summary Table	35

1. Preamble

This document defines annotations (i.e., image features such as points, lines and edge contours) within anteroposterior (AP) or posteroanterior (PA) radiographic/fluoroscopic images. It is intended to provide a consistent reference for annotation placement within the annotation tool in order to define a dataset for downstream machine learning tasks.

Each annotation definition includes:

- the identifying annotation IDs;
- a definition of the annotation;
- optional further notes relevant to interpretation or repeatability; and
- visual examples.

The document includes the following sections:

- **Medical Approval:** Records review of the document for medical appropriateness and clinical usability.
- **Image Annotations:** Defines the points, lines and contours to be placed directly by annotators.
- **Annotation Order:** Specifies the recommended sequence for completing annotations so that prerequisite anatomical references are established before dependent annotations are placed.
- **General Guidance for Contour Annotation:** Explains what a contour is, how to trace a visible boundary and how to handle contours that are partly obscured or extend beyond the image.
- **Derived Features of Interest:** Defines points, lines and other measurements that are deterministically calculated from one or more source annotations and subsequently presented to the annotator for approval.
- **Categorical Annotation:** Defines the image-level categorical classifications that annotators are required to assign.
- **Annotation ID Summary Table:** Provides a consolidated reference for the annotation identifiers used throughout the document.

Derived features are not placed independently by the annotator. They are calculated automatically from the accepted source annotations according to the definitions provided in this document. Annotators are responsible for reviewing the generated features and categorising each as correct, incorrect or missing, as applicable.

Unless otherwise stated, left- and right-sided annotations use equivalent definitions reflected across the image. References to superior, inferior, medial, lateral, proximal and distal directions are anatomical references and should be interpreted with respect to the patient rather than the displayed orientation of the image.

All annotations must be selected according to the visible features present in the image under review. For directly traced contours, annotators must stop at the last clearly visible point and restart when the boundary becomes visible again, leaving a gap across any obscured region. Annotators should not infer the position of an anatomical boundary through a region in which that boundary is not visible unless the applicable annotation definition explicitly instructs them to fit or approximate a geometric structure.

When an annotation is visible but its exact placement cannot be determined consistently, the annotator should flag it for further review and may include a note describing the source of uncertainty. An annotation which is clearly not visible, not present or not applicable should instead be recorded using the corresponding missing or non-applicable status available within the annotation tool.

Where the annotation tool automatically constrains, fits or derives a feature, the resulting feature must still satisfy the anatomical and geometric definition given in this document. Tool-generated placement does not replace annotator review or the approval criteria specified for the feature.

2. Medical Approval

This annotation definition document has been reviewed for medical appropriateness and clinical usability.

Approved by:

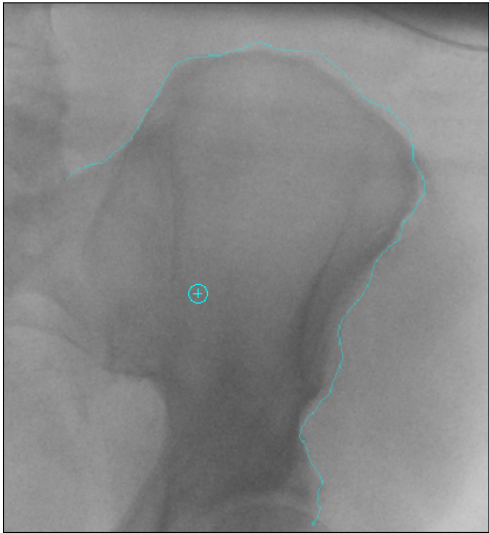
Surgeon Name: _____ **Date:** _____

Signature: _____ **Institution / Department:** _____

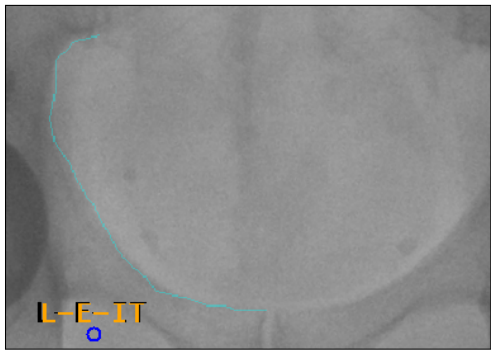
Comments / approval notes:

3. Image Annotations

3.1. Ilium Contour

Ilium Contour	
Annotation IDs	L-C-ILI Left Superior-Lateral Ilium Contour
	R-C-ILI Right Superior-Lateral Ilium Contour
Definition	The contour that traces from the edge of the spine along the sacrum and ilium down to the acetabular rim. Although the annotation is named after the ilium, portions of the contour mark the superior boundary of the sacrum.
Further Notes	Annotate only the visible portion of the boundary. If the contour is obscured, stop at the last clearly visible point and restart when the boundary becomes visible again, leaving a gap across the obscured region. Do not visually interpolate across an occlusion. If the contour continues beyond the image, stop at the image boundary.
Visual Example(s)	 An example of a left ilium contour.

3.2. Pelvic Outlet Contour

Pelvic Outlet Contour	
Annotation IDs	L-C-P0 Left Pelvic Outlet Contour
	R-C-P0 Right Pelvic Outlet Contour
Definition	The contour that traces the boundary of the pelvic outlet from the superior margin of the pubic symphysis to the distal sacroiliac joint.
Further Notes	Annotate only the visible portion of the boundary. If the contour is obscured, stop at the last clearly visible point and restart when the boundary becomes visible again, leaving a gap across the obscured region. Do not visually interpolate across an occlusion. If the contour continues beyond the image, stop at the image boundary.
Visual Example(s)	 An example of a right pelvic outlet contour.

3.3. Pelvic Teardrops

Pelvic Teardrops

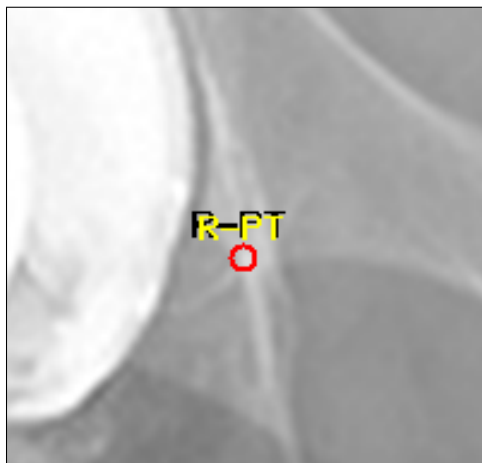
Annotation IDs

L-PT	Left Pelvic Teardrop
R-PT	Right Pelvic Teardrop

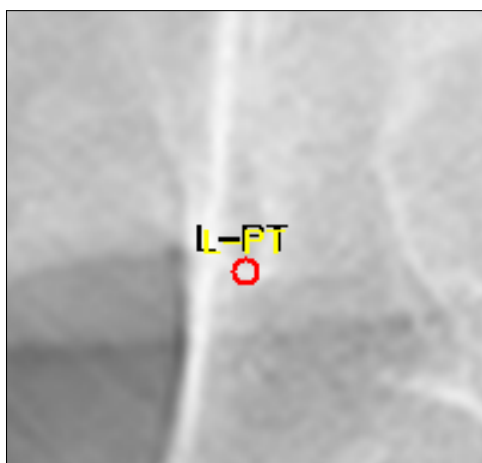
Definition

Select the most inferior edge at the bottom of the teardrop.

Visual Example(s)

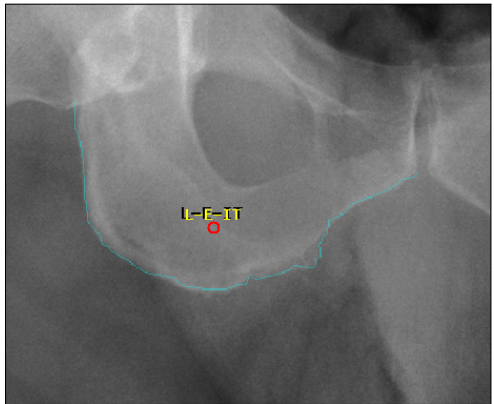


An example of a right pelvic teardrop landmark near an implant.

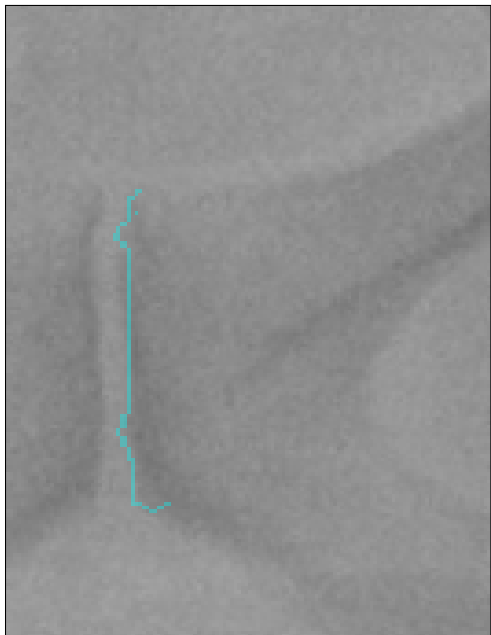


An example of a left pelvic teardrop landmark.

3.4. Ischial Tuberosities

Ischial Tuberosities	
Annotation IDs	L-C-IT Left Ischial Tuberosity Contour
	R-C-IT Right Ischial Tuberosity Contour
Definition	The contour that traces the inferior boundary of the ischial tuberosity from the inferior margin of the pubic symphysis to the point at which the lateral ischium meets the acetabular rim. If the femur obscures this boundary before that endpoint is reached, stop at the last clearly visible point. The contour must not trace along an ischial boundary that overlaps with the femur.
Further Notes	The point at which the lateral ischium meets the acetabular rim is the superior-lateral endpoint of this contour. Annotate only the visible portion of the boundary. If the contour is obscured, stop at the last clearly visible point and restart when the boundary becomes visible again, leaving a gap across the obscured region. Do not visually interpolate across an occlusion. If the contour continues beyond the image, stop at the image boundary.
Visual Example(s)	 An example of a right ischial tuberosity contour.

3.5. Pubic Symphysis Contour

Pubic Symphysis Contour	
Annotation IDs	L-C-PS Left Pubic Symphysis Contour
	R-C-PS Right Pubic Symphysis Contour
Definition	The contour that spans one side of the pubic symphysis from its superior margin to its inferior margin; that is, between the apparent corner points formed by the superior and inferior margins of the pubis along the border of the pubic symphysis. If both internal and external contours are visible, choose the most medial contour.
Further Notes	Annotate only the visible portion of the boundary. If the contour is obscured, stop at the last clearly visible point and restart when the boundary becomes visible again, leaving a gap across the obscured region. Do not visually interpolate across an occlusion. If the contour continues beyond the image, stop at the image boundary.
Visual Example(s)	 An example of a right pubic symphysis contour.

3.6. Distal Sacroiliac Joint

Distal Sacroiliac Joint

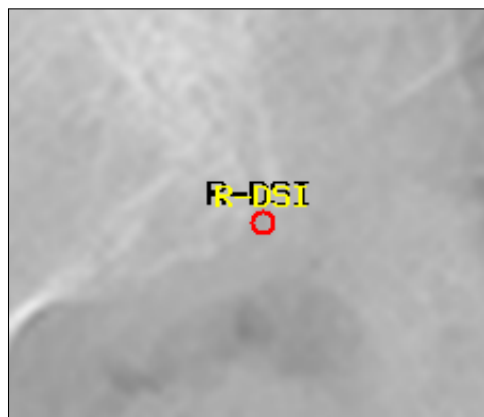
Annotation IDs

L-DSI	Left Distal Sacroiliac Joint
R-DSI	Right Distal Sacroiliac Joint

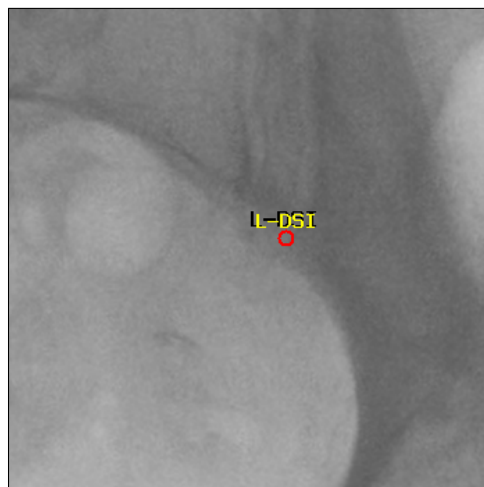
Definition

Select the point at the visible distal termination of the sacroiliac (SI) joint between the visible bone edges (centre of the gap). If multiple joint lines are visible, choose the most lateral.

Visual Example(s)

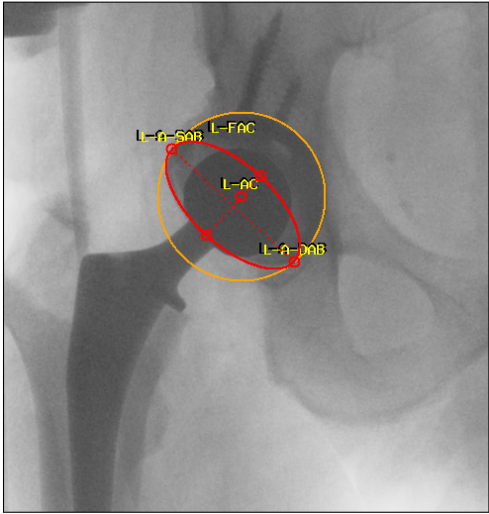


An example of a right distal sacroiliac joint landmark.

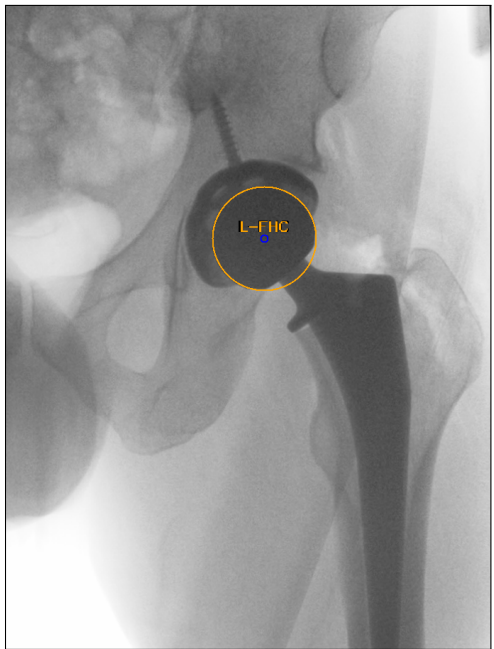


An example of a correctly selected left distal sacroiliac joint landmark in the presence of two apparent joint lines.

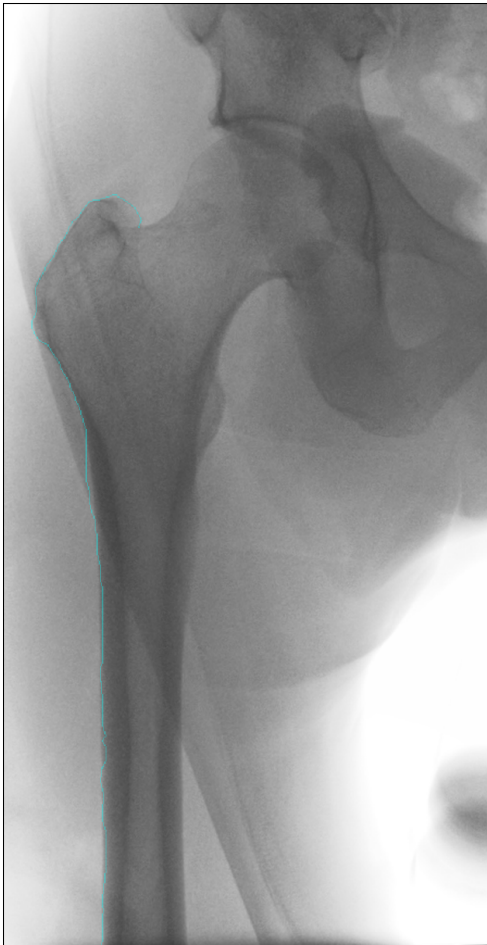
3.7. Acetabular Contour

Acetabular Contour	
Annotation IDs	L-C-AC Left Acetabular Contour
	R-C-AC Right Acetabular Contour
	L-C-FAC Left Acetabular Face Contour
	R-C-FAC Right Acetabular Face Contour
Definition	For an acetabular implant, define both the spherical contour of the cup and the elliptical contour of its face. Place the spherical contour first using the hover circle tool , fitting it to the spherical portion of the acetabular cup implant. Then place the elliptical contour using the acetabular face tool , which snaps the major-axis endpoints to the fitted circle. Adjust the inclination by dragging the major-axis endpoints around the circle, and expand or shrink the minor axis to match the visible contour of the implant face, typically around its inferior portion. Do not annotate either the acetabular spherical contour or the acetabular face contour for a native hip.
Further Notes	Apply the geometric fit only when enough of the relevant boundary is visible to position it reliably. If the available boundary is insufficient, record the annotation as missing rather than estimating its placement from unrelated anatomy.
Visual Example(s)	 An example of a right acetabular spherical contour and elliptical face contour.

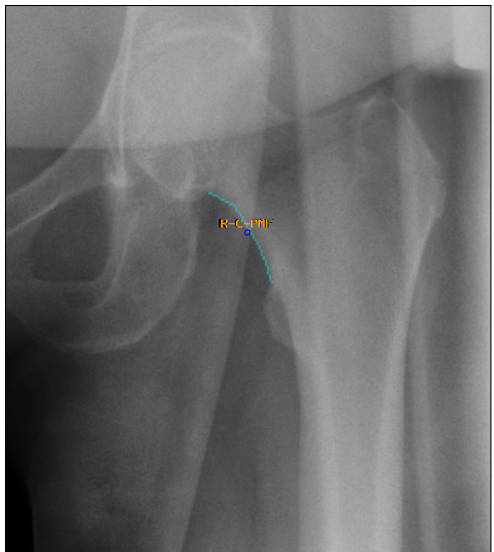
3.8. Femoral Head Contour

Femoral Head Contour	
Annotation IDs	L-C-FHC Left Femoral Head Contour
	R-C-FHC Right Femoral Head Contour
Definition	For a native hip, use the hover circle to approximate the contour of the visible bony femoral head. For a prosthetic hip, fit the hover circle to the outer margin of the femoral head implant.
Further Notes	Apply the geometric fit only when enough of the relevant boundary is visible to position it reliably. If the available boundary is insufficient, record the annotation as missing rather than estimating its placement from unrelated anatomy.
Visual Example(s)	 An example of a left femoral head contour on an implant.

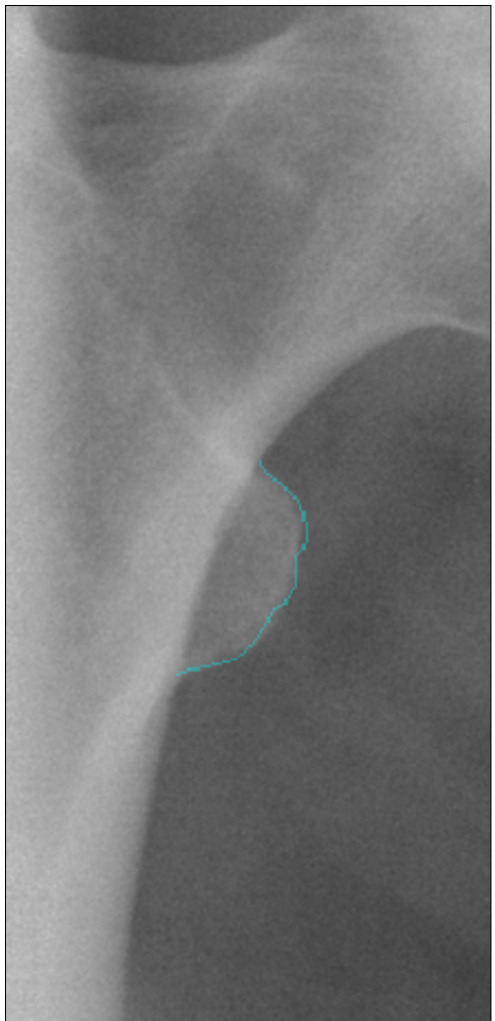
3.9. Greater Trochanter Contour

Greater Trochanter Contour	
Annotation IDs	L-C-GT Left Greater Trochanter Contour
	R-C-GT Right Greater Trochanter Contour
Definition	The contour along the superior-lateral boundary of the femur from the neck-greater trochanter transition point to the most distal visible lateral point in the image.
Further Notes	Annotate only the visible portion of the boundary. If the contour is obscured, stop at the last clearly visible point and restart when the boundary becomes visible again, leaving a gap across the obscured region. Do not visually interpolate across an occlusion. If the contour continues beyond the image, stop at the image boundary.
Visual Example(s)	 An example of a right greater trochanter contour.

3.10. Proximal Medial Femur Contour

Proximal Medial Femur Contour	
Annotation IDs	L-C-PMF Left Proximal Medial Femur Contour
	R-C-PMF Right Proximal Medial Femur Contour
Definition	The contour along the medial boundary of the proximal femur from the proximal tip of the lesser trochanter boundary up the neck to the femoral head.
Further Notes	Annotate only the visible portion of the boundary. If the contour is obscured, stop at the last clearly visible point and restart when the boundary becomes visible again, leaving a gap across the obscured region. Do not visually interpolate across an occlusion. If the contour continues beyond the image, stop at the image boundary.
Visual Example(s)	 An example of a left proximal medial femur contour.

3.11. Lesser Trochanter Contour

Lesser Trochanter Contour	
Annotation IDs	L-C-LT Left Lesser Trochanter Contour
	R-C-LT Right Lesser Trochanter Contour
Definition	The contour along the visible medial boundary of the lesser trochanter, beginning at its proximal transition from the medial femur and ending at its distal transition back to the medial femoral shaft.
Further Notes	Annotate only the visible portion of the boundary. If the contour is obscured, stop at the last clearly visible point and restart when the boundary becomes visible again, leaving a gap across the obscured region. Do not visually interpolate across an occlusion. If the contour continues beyond the image, stop at the image boundary.
Visual Example(s)	
An example of a right lesser trochanter contour.	

3.12. Distal Medial Femur Contour

Distal Medial Femur Contour	
Annotation IDs	L-C-DMF Left Distal Medial Femur Contour
	R-C-DMF Right Distal Medial Femur Contour
Definition	The contour along the medial boundary of the femur from the distal tip of the lesser trochanter to the most distal visible medial point in the image.
Further Notes	Annotate only the visible portion of the boundary. If the contour is obscured, stop at the last clearly visible point and restart when the boundary becomes visible again, leaving a gap across the obscured region. Do not visually interpolate across an occlusion. If the contour continues beyond the image, stop at the image boundary.

Distal Medial Femur Contour (Continued)

Visual Example(s)



An example of a left distal medial femur contour.

3.13. Obturator Foramen Contour

Obturator Foramen Contour

Annotation IDs

L-C-OF	Left Obturator Foramen Contour
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R-C-OF	Right Obturator Foramen Contour
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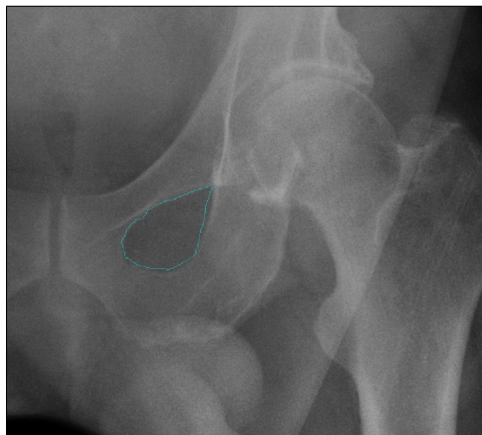
Definition

The contour along the visible boundary of the obturator foramen.

Further Notes

Annotate only the visible portion of the boundary. If the contour is obscured, stop at the last clearly visible point and restart when the boundary becomes visible again, leaving a gap across the obscured region. Do not visually interpolate across an occlusion. If the contour continues beyond the image, stop at the image boundary.

Visual Example(s)



An example of a left obturator foramen contour.

4. Annotation Order

To support consistent annotation placement, the image annotations should be completed in the order listed below. The sequence is shown for the left side only; the equivalent right-sided image annotations should be completed in the same relative order.

The order is intended to establish clear anatomical reference points before annotations which depend on those points, to complete related pelvic structures together, and to ensure that shared anatomical transition points are defined consistently.

The thirteen numbered items below correspond to the thirteen landmark groups defined in Section 3. The acetabular landmark group contains two separate annotation objects, the spherical acetabular contour and the acetabular face contour, which must be completed sequentially. Derived features, including the femoral axis, are not manually annotated and therefore do not appear as separate items in this ordering.

1. **Left Ilium Contour** (**L-C-ILI**)

Annotate the ilium contour first to establish the broad superior and lateral boundary of the hemipelvis.

2. **Left Pubic Symphysis Contour** (**L-C-PS**)

Annotate the pubic symphysis contour before the pelvic outlet and ischial tuberosity contours. Its superior and inferior margins establish the anatomical transition points from which those contours begin.

3. **Left Distal Sacroiliac Joint** (**L-DSI**)

Select the distal termination of the sacroiliac joint before drawing the pelvic outlet contour. This point defines the distal sacroiliac endpoint of the pelvic outlet contour and reduces variation in where that contour is terminated.

4. **Left Pelvic Outlet Contour** (**L-C-PO**)

Draw the pelvic outlet contour after the pubic symphysis and distal sacroiliac joint have been identified, since these annotations establish the anatomical endpoints of the contour.

5. **Left Obturator Foramen Contour** (**L-C-OF**)

Annotate the obturator foramen after the principal adjacent pelvic boundaries have been established. Although this contour does not depend directly on another annotation, completing it at this stage keeps the central pelvic annotations together and provides additional anatomical context for the remaining pelvic structures.

6. **Left Ischial Tuberosity Contour** (**L-C-IT**)

Annotate the ischial tuberosity contour after the pubic symphysis has been defined. The contour begins at the inferior margin of the pubic symphysis and continues along the visible inferior boundary of the ischial tuberosity.

7. **Left Pelvic Teardrop** (**L-PT**)

Select the pelvic teardrop after the surrounding pelvic contours have been annotated. Although the teardrop does not depend geometrically on those contours, the surrounding annotations provide useful anatomical context for consistent placement.

8. **Left Acetabular Contours** (**L-C-AC** and **L-C-FAC**)

For a prosthetic hip, first fit the spherical acetabular contour (**L-C-AC**) to the spherical portion of the acetabular cup implant. Then place the acetabular face contour (**L-C-FAC**). The acetabular face tool is constrained by the fitted spherical contour: its major-axis endpoints are snapped to the acetabular circle. The spherical contour must therefore be correctly positioned before the face ellipse is adjusted.

Neither acetabular contour should be annotated for a native hip.

9. **Left Femoral Head Contour** (**L-C-FHC**)

Fit the femoral head contour after completing any applicable acetabular annotations. This establishes the femoral head geometry before the remaining proximal femoral boundaries are traced.

10. **Left Greater Trochanter Contour** (**L-C-GT**)

Annotate the greater trochanter contour after the femoral head has been defined. The greater trochanter contour should extend from the neck-greater trochanter transition to the most distal visible lateral point of the femur.

This contour also contributes to the subsequently derived femoral axis.

11. **Left Lesser Trochanter Contour** (**L-C-LT**)

Annotate the lesser trochanter contour before the proximal and distal medial femur contours. Trace the visible medial boundary of the lesser trochanter from its proximal transition with the medial femur to its distal transition back to the medial femoral shaft.

Completing this contour first establishes the proximal and distal lesser trochanter transition points used to define the adjacent medial femoral contours.

12. **Left Proximal Medial Femur Contour** (**L-C-PMF**)

Trace the proximal medial femoral boundary from the proximal transition of the lesser trochanter toward the femoral head. The lesser trochanter contour should already be complete so that the shared transition between the two annotations is identified consistently.

13. **Left Distal Medial Femur Contour** (**L-C-DMF**)

Begin the distal medial femur contour at the distal transition of the lesser trochanter and continue along the medial femoral boundary to the most distal visible medial point in the image.

The lesser trochanter contour should already be complete so that the shared transition is identified consistently. Completion of this contour, together with the greater trochanter contour, provides the source contours required for calculation of the derived femoral axis.

General Ordering Principles

- Complete all image annotations for one side before beginning the opposite side.
- Establish anatomical reference points and shared transition points before drawing contours which begin or end at those locations.
- Skip annotations which are not applicable, such as acetabular implant contours on a native hip.
- Derived features are generated automatically from their source annotations and should be reviewed against their defined approval criteria rather than manually placed.

5. General Guidance for Contour Annotation

5.1. What Is a Contour?

A **contour** describes the visible outer boundary of an anatomical structure, implant or instrument in the two-dimensional image. For model-training purposes, a continuous contour is converted into a discrete set of pixels at the resolution supplied to neural networks. A pixel belongs to the discrete contour set when the continuous contour intersects the area assigned to that pixel. For boundary cases, all four edges of a pixel are considered as belonging to that pixel (for example, a perfectly straight line on the boundary between two pixels would count both of those pixels as part of the discrete contour set).

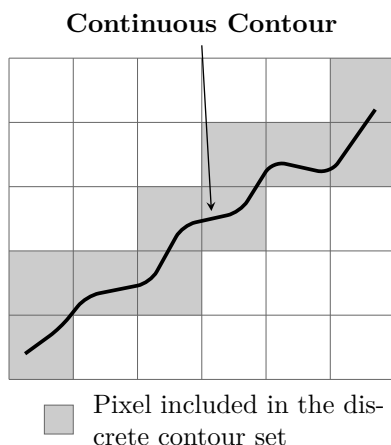


Figure 1: Illustration of discrete contour membership.

5.2. How to Annotate a Contour

- Follow the visible edge of the structure as closely as possible.
- The annotation tool will automatically highlight regions at the level of resolution supplied to the neural networks.
- Do not trace the edge of an overlapping implant, instrument, soft-tissue structure or different bone unless the definition explicitly instructs you to do so.

5.3. Partly Obscured or Off-Screen Contours

Annotate only the portions of the boundary that are clearly visible. If the contour is hidden by an implant, surgical instrument, table hardware, another anatomical structure or any other object, stop at the last clearly visible point and restart when the same boundary becomes visible again. Leave the hidden section blank rather than visually interpolating across it. A single anatomical contour may therefore contain two or more separate annotated segments.

If the contour continues beyond the edge of the image, stop at the image boundary. Do not estimate the missing portion. If the missing section prevents a downstream feature from being

calculated correctly, classify that downstream feature as *incorrect* or *missing*, as appropriate, during categorical review.

6. Derived Features of Interest

Many of the contour annotations have associated features of interest which are deterministically calculated from the contour set. These will be auto-generated by the annotation tool and annotators should ensure that these downstream features meet the terms of their definitions (defined in the tables below).

Contour ID	Source Contour	No.	Derived Feature ID / Name	Definition and Approval Criteria
C-ILI	Superior-Lateral Ilium Contour	1	SIP Superior Ilium Point	The most superior point on the ilium contour. If more than one contour point has the same superior-most vertical position, select the most lateral of the tied points. The principal downstream value of interest is the vertical coordinate of the selected point.
		2	LIP Lateral Innominate Point	The most lateral point on the ilium contour. If more than one contour point has the same lateral-most horizontal position, select the most superior of the tied points. The principal downstream value of interest is the horizontal coordinate of the selected point.

Contour ID	Source Contour	No.	Derived Feature ID / Name	Definition and Approval Criteria
C-PO	Pelvic Outlet Contour	1	LP0 Lateral Pelvic Outlet Point	The most lateral point on the pelvic outlet contour. If more than one contour point has the same lateral-most horizontal position, select the point at the median vertical position among the tied points. The principal downstream value of interest is the horizontal coordinate of the selected point.
		2	POD Pelvic Outlet Diameter	The line segment between the two lateral pelvic outlet points.

Contour ID	Source Contour	No.	Derived Feature ID / Name	Definition and Approval Criteria
C-IT	Ischial Tuberosity Contour	1	IT Inferior Ischial Tuberosity Point	The most inferior point on the ischial tuberosity contour. If more than one contour point has the same inferior-most vertical position, select the most lateral of the tied points. The principal downstream value of interest is the vertical coordinate of the selected point.

Contour ID	Source Contour	No.	Derived Feature ID / Name	Definition and Approval Criteria
C-PS	Pubic Symphysis Contour	1	SPS Superior Pubic Symphysis Point	The most proximal, or superior, point on the pubic symphysis contour. If more than one contour point has the same proximal-most vertical position, select the most medial of the tied points. The principal downstream value of interest is the vertical coordinate of the selected point.
		2	IPS Inferior Pubic Symphysis Point	The most distal, or inferior, point on the pubic symphysis contour. If more than one contour point has the same distal-most vertical position, select the most medial of the tied points. The principal downstream value of interest is the vertical coordinate of the selected point.

Contour ID	Source Contour	No.	Derived Feature ID / Name	Definition and Approval Criteria
C-AC + C-FAC	Acetabular Contour and Acetabular Face Contour	1	DAB Distal Acetabular Boundary Point	The more medial of the two intersection points between the fitted acetabular circle and the fitted acetabular face ellipse. This point must coincide with both fitted contours and defines the medial endpoint of the major axis of the acetabular face ellipse.
		2	SAB Superior Acetabular Boundary Point	The more lateral of the two intersection points between the fitted acetabular circle and the fitted acetabular face ellipse. This point must coincide with both fitted contours and defines the lateral endpoint of the major axis of the acetabular face ellipse.

Contour ID	Source Contour	No.	Derived Feature ID / Name	Definition and Approval Criteria
C-AC	Acetabular Contour	1	AC Acetabular Centre	The centre of the circle fitted to the spherical acetabular contour. The generated point should be approved only when it is the geometric centre of the accepted acetabular circle.

Contour ID	Source Contour	No.	Derived Feature ID / Name	Definition and Approval Criteria
C-FHC	Femoral Head Contour	1	FHC Femoral Head Centre	The centre of the circle fitted to the femoral head contour. The generated point should be approved only when it is the geometric centre of the accepted femoral head circle.

Contour ID	Source Contour	No.	Derived Feature ID / Name	Definition and Approval Criteria
C-GT	Greater Trochanter Contour	1	SGT Superior Greater Trochanter Point	The most superior point on the greater trochanter contour. If more than one contour point has the same superior-most vertical position, select the most lateral of the tied points.
		2	LGT Lateral Greater Trochanter Point	The most lateral point on the greater trochanter contour. If more than one contour point has the same lateral-most horizontal position, select the most superior of the tied points.

Contour ID	Source Contour	No.	Derived Feature ID / Name	Definition and Approval Criteria
C-LT	Lesser Trochanter Contour	1	PLT Proximal Lesser Trochanter Point	The most proximal point on the lesser trochanter contour. If more than one contour point has the same proximal-most vertical position, select the most lateral of the tied points.
		2	MLT Medial Lesser Trochanter Point	The most medial point on the lesser trochanter contour. If more than one contour point has the same medial-most horizontal position, select the most superior of the tied points.
		3	DLT Distal Lesser Trochanter Point	The most distal point on the lesser trochanter contour. If more than one contour point has the same distal-most vertical position, select the most lateral of the tied points.

Contour ID	Source Contour	No.	Derived Feature ID / Name	Definition and Approval Criteria
C-DMF + C-GT	Distal Medial Femur Contour and Greater Trochanter Contour	1	FA Derived Femoral Axis	Determine the proximal third of the distal medial femur contour using its vertical extent. Restrict the greater trochanter contour to the corresponding vertical range. Use the two resulting contour segments to fit a central line which approximately bisects the medial and lateral boundaries. The resulting line is a candidate proxy for the femoral shaft axis. It should be approved only when it lies centrally between the two contour segments throughout the fitted vertical range and is not visibly biased towards either boundary.

7. Categorical Annotation

In addition to the image annotations defined in Section 3, annotators should make the following categorical annotations regarding image features.

1. For each side (left and right), is the hip native, prosthetic, or other (e.g., tooling is present)?
2. For each annotation, is every associated downstream feature of interest correct, incorrect, or missing?

A. Annotation ID Summary Table

Annotation ID	Definition
L-C-ILI	Left Superior-Lateral Ilium Contour
R-C-ILI	Right Superior-Lateral Ilium Contour
L-C-PO	Left Pelvic Outlet Contour
R-C-PO	Right Pelvic Outlet Contour
L-PT	Left Pelvic Teardrop
R-PT	Right Pelvic Teardrop
L-C-IT	Left Ischial Tuberosity Contour
R-C-IT	Right Ischial Tuberosity Contour
L-C-PS	Left Pubic Symphysis Contour
R-C-PS	Right Pubic Symphysis Contour
L-DSI	Left Distal Sacroiliac Joint
R-DSI	Right Distal Sacroiliac Joint
L-C-AC	Left Acetabular Contour
R-C-AC	Right Acetabular Contour
L-C-FAC	Left Acetabular Face Contour
R-C-FAC	Right Acetabular Face Contour
L-C-FHC	Left Femoral Head Contour
R-C-FHC	Right Femoral Head Contour
L-C-GT	Left Greater Trochanter Contour
R-C-GT	Right Greater Trochanter Contour
L-C-PMF	Left Proximal Medial Femur Contour
R-C-PMF	Right Proximal Medial Femur Contour
L-C-LT	Left Lesser Trochanter Contour
R-C-LT	Right Lesser Trochanter Contour
L-C-DMF	Left Distal Medial Femur Contour
R-C-DMF	Right Distal Medial Femur Contour
L-FA	Left Femoral Axis
R-FA	Right Femoral Axis
L-C-OF	Left Obturator Foramen Contour
R-C-OF	Right Obturator Foramen Contour